

CILE Clinical Consultation Report

Patient Vitals & Demographics

Age: 72 | Gender: M
Weight: 80.0 kg | SrCr: 1.8 mg/dL
Estimated CrCl: 41.98 mL/min

Checking for DRUG-DRUG INTERACTIONS

[CRITICAL] FUROSEMIDE & IBUPROFEN: [RENAL] NSAIDs blunt the diuretic and antihypertensive efficacy of loop diuretics. Increased risk of renal impairment.

[CRITICAL] LISINOPRIL & IBUPROFEN: [CRITICAL] Renal hemodynamics compromised. ACEi prevents efferent arteriole constriction while NSAID prevents afferent dilation. High risk of Acute Kidney Injury (AKI).

[CRITICAL] LISINOPRIL & FUROSEMIDE: [MONITORING] Potential for profound hypotension and renal impairment if patient is volume depleted. Monitor SrCr and BP.

Predictive Clinical Alerts & Profiles

Drug: Ibuprofen

[Drug Classification]: BEERS, RENAL
[RoA]: Oral
[PGx Markers Found] Targets: None identified

> [CLINICAL INTERCEPT] Risk of peptic ulcer disease and renal impairment; avoid chronic use in patients with eGFR <60.

Drug: Furosemide

[Drug Classification]: RENAL, OTOTOXIC
[RoA]: IV, Oral
[PGx Markers Found] Targets: RECEPTOR

> [CLINICAL INTERCEPT] Potent loop diuretic. Risk of profound diuresis, electrolyte depletion, and ototoxicity (especially with rapid IV push).

> [FDA ALERT] The renal clearance of furosemide after intravenous administration in older healthy male subjects is statistically significantly smaller than in younger healthy male subjects

> [FDA ALERT] In general, dose selection for the elderly patient should be cautious, usually starting at the low end of the dosing range, reflecting the greater frequency of decreased hepatic, renal or cardiac function, and of concomitant disease or other drug therapy

> [FDA ALERT] This drug is known to be substantially excreted by the kidney, and the risk of toxic reactions to this drug may be greater in patients with impaired renal function.

Drug: Lisinopril

[Drug Classification]: UNCLASSIFIED
[RoA]: IV, Oral, SubQ
[PGx Markers Found] Targets: RECEPTOR

> [FDA ALERT] In general, dose selection for an elderly patient should be cautious, usually starting at the low end of the dosing range, reflecting the greater frequency of decreased hepatic, renal, or cardiac function, and of concomitant disease or other drug therapy

> [FDA ALERT] This drug is known to be substantially excreted by the kidney, and the risk of toxic reactions to this drug may be greater in patients with impaired renal function

> [FDA ALERT] Because elderly patients are more likely to have decreased renal function, care should be taken in dose selection.